3.8 Pleural Effusion*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot take a deep breath." **Objective** • Less chest movement on the affected side • Dull percussion note • No breath sounds at the base; SpO_2 91%	**ACTUAL** Ineffective breathing pattern related to reduced lung expansion from pleural fluid as evidenced by dyspnea and reduced breath sounds.	• After drainage, RR 12–20/min and SpO_2 94% or above.	• Sit her upright. • Give oxygen as prescribed. • Prepare for thoracentesis: explain it and sit her leaning forward over a table. • Watch vital signs and SpO_2 during and after the procedure. • Record how much fluid is removed and what it looks like.	• Achieved when breathing is easier after drainage.
Subjective • "The side of my chest hurts when I breathe." **Objective** • Guarding • Shallow breathing	**ACTUAL** Acute pain related to pleural inflammation as evidenced by pleuritic pain and shallow respirations.	• Pain at or below 3/10 within an hour of analgesia.	• Give analgesia as prescribed and reassess after 30–60 minutes. • Show her how to hold a pillow against the chest when coughing. • Let her lie on the affected side if that helps. • Once pain is controlled, teach slow deep breathing.	• Outcome reached when pain is under control.
Subjective *No specific complaint* **Objective** • After thoracentesis	**RISK** Risk for physical injury as evidenced by risk factors: invasive procedure (pneumothorax, hemorrhage, re-expansion pulmonary edema).	• No complications after the procedure.	• Watch for sudden breathlessness, chest pain or a fall in SpO_2. • Arrange a chest X-ray as ordered. • Check the puncture site for bleeding.	• Considered met if no complications arise.

*References for this care plan: [17], [32], [35], [37]. Full entries are listed in the References section of the CPMN book.